جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : حسام احمد خليل خليل

المحافظة : الشرقيه **الحي / المدينة :** الحسينيه **العنوان :** ش

رقم البطاقة : 28512121303431 **رقم التليفون :** 01555082140 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 12-12-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان البنكرياس

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان البنكرياس

خطة العلاج : أورام كيماوي خط علاج كيمائي اول ما قبل او بعد الجراحه طبقا لبروتوكول (Folfoirnox) [14229] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: حسام احمد خليل خليل
National ID: 28512121303431

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of pancreatic cancer for FOLFIRINOX Chemotherapy Protocol

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Date Of Submission: Date Of MDT: 08/13/2025 15:45:42

GUSTAVE ROUSSY INTERNATIONAL ACADEMY

Medical Report

Patient Name :	[illegible]	Age :	41 Y
Sex :	◉ Male ○ Female	Comorbidity :	DM
ID No. :	13250	Telephone :	01555021027
Family History :	Negative	Marital Status :	◉ Married ○ Divorced ○ Single
Menopausal Status :		Reason of MDT :	New Case
		MDT number :	

Medical Report

Diagnosis : MDA. Diff adenocarcinoma body and tail of Pancreas

Notes :
41 ys male /ex smoker , diagnosed in late july 2025 by Cancer Tail body of Pancreas in MSCT AP,, : Endoscopic Biopsy : Mod Diff Adenocarcinoma. Borderline Resectable ,, For MDT to start NACT and Surgical evaluation of resectability

History : ECOG PS 1

Investigations : CT PA : Hypodense mass lesion in the body and tail of pancreas ,encircling splenic artery,, Inseparable surrounding fat stranding from mass (local spread)
CT Chest : Bil small lung nodules for F/U Scan
EUS biopsy from lesion: Mod diff adenocarcinoma
CA 19-9 256

Recommendations : For Radiologists/Surgical assessment of imaging to confirm resectability and metastatic status.
MRI PA +/- PET-CT if in need
If confirmed, borderline resectable to start NACT mFolfirinox / Folfeoxiri and then surgery acc to response
Pain Clinic consultation/ Nutrition consultation

MDT DECISION: For ECHO , Fortacath
-->borderline resectable to start NACT Folfirinox (4 cycles each with day 1, day 15) / and then assess resectability with CT Chest and PA + Pancreatic Protocol and Surgery acc to response
Pain Clinic consultation/ Nutrition consultation

Specialty	Surgery	Pathology	Radiology

GUSTAVE ROUSSY INTERNATIONAL

Date Of Submission:

Date Of MDT: 09/17/2026 15:23:13

Medical Report

Patient Name :	حسام احمد خليل خليل	**Age :**	41 Y
Sex :	☐ Male ◯ Female	**Comorbidity :**	no
ID No. :	13200	**Telephone :**	01555071077
Family History :	Negative	**Marital Status :**	◉ Married ◯ Divorced ◯ Single
Menopausal Status :		**Reason of MDT :**	New Case
		MDT name::	GIT cancers

Medical Report

Diagnosis : MOd. Diff adenocarcinoma body and tail of Pancreas

Notes :

41 yo male non smoker , diagnosed in late july 2026 by Cancer Tail/body of Pancreas in MSCT AP... :Endoscopic Biopsy : Mod Diff Adenocarcinoma. Borderline Resectable .. For MDT to start NACT and Surgical evaluation of resectability

History:	ECOG PS 1
Investigations:	CT PA :Hypodechsic mass lesion in the body and tail of pancreas, abutting splenic artery,, inseperable surrounding fat planes from mass (local spread) CT Chest : Bil small lung adules for FU 5mm EUS biopsy from lesion : Mod diff adenocarcinoma CA 19-9 289
Recommendations:	For Radiodiagnosis/Surgical assessment of imagings to confirm resectability and metastatic status. MRI PA +- PET-CT if in need If confirmed, borderline resectable to start NACT mFolfirinox / Portacath and then surgery acc to response Pain Clinic consultation/ Nutrition consultation
MDT DECISION:	For ECHO , Portacath -->borderline resectable to start NACT Folfirinox (4 cycles each with day 1, day 15) / and then assess resectability with CT Chest and PA e Pancreatic Protocol and Surgery acc to response Pain Clinic consultation/ Nutrition consultation

Speciality	Surgery	Pathology	Radiology
Medical oncology			

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SCAN

Name: Hossam Ahmed Khalil Khalil *ID: 2168429*
Referred by: Prof. Yasser Al-Debeky *Date: 06/08/2026*

ECHOCARDIOGRAPHY REPORT

DIMENSIONS

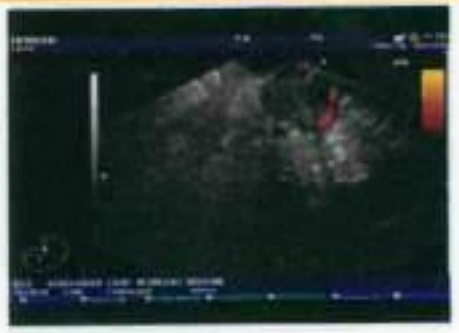
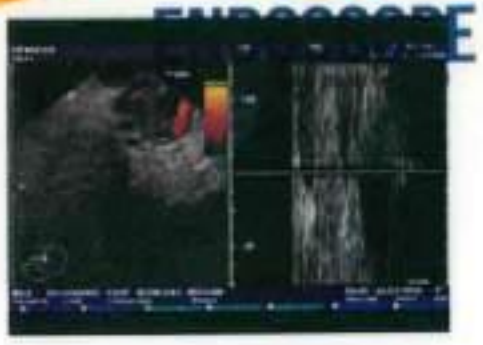
LVEDD	4.9	(<5.6 cm)	**FS**	27% (>31%)
LVESD	3.6		**EF**	53% (>55%)
IVSWT	0.9	(<1.1 cm)	**MITRAL FLOW**	NORMAL
PWT	0.8	(<1.1 cm)	**AORTIC FLOW**	NORMAL
AO ROOT	3.1	(<3.7 cm)	**PULMONARY FLOW**	NORMAL
LA	3.4	(<4.0 cm)	**TRICUSPID FLOW**	NORMAL
RV	1.9	(<2.7 cm)		

CONCLUSIONS:

The basal HR=110 bpm

- Normal left ventricular internal dimensions, and wall thickness with **good LV global systolic function (EF= 53% by m-mode).**
- Antegrade diastolic mitral flow shows fused E and A waves **(sinus tachycardia).**
- No regional wall motion abnormality at rest apart from **beginning of abnormal (paradoxical) septal wall motion.**
- Trivial mitral regurge.
- Trivial aortic regurge.
- Normal other valvular aspect, thickness and function.
- Normal left atrium. Normal aortic root dimensions.
- Trivial tricuspid regurge with no indirect signs of pulmonary hypertension. PASP=26+CVP mmHg, with normal dimensions and contractility of the right sided chambers.
- No detected intra-cardiac shunts by TTE.
- No detected intra-cardiac masses or thrombi.
- Normal pericardium.

Best Regards,
Rania El-Husseiny, MD.





ENDOSCOPE
الدوسكوب لتجهاز الهضمي والمناظير

EUS Report

Patient Name:	حسام احمد خليل	**ID**	32846
Endoscopist:	Dr Ahmed Ramadan	**DOB**	
2nd Operator		**Age:**	40 Y
Referred By:	-	**Date**	30/07/2026

INDICATION FOR EXAMINATION

PROCEDURE PERFORMED

Diagnostic

MEDICATIONS

FINDINGS

Pancreas:

There was solid hypoechoic pancreatic mass about 24mmx23mm at body ,tail junction 22FNA g needl biopsy was done.

Mass was abuting splenic vien away from portal confluence,displaying and streching splenic artery.

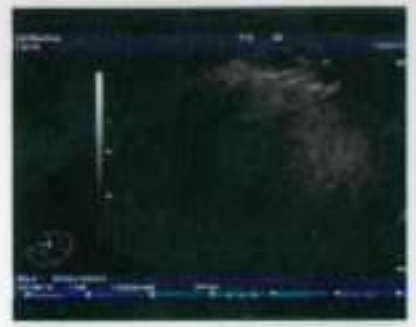
Surrounding panchreatic parenchyma showed hony combing .

Pancreatic duct was not dilated.

CBD was not dilated.

No prelesiona LNS.

No IHBRD






Conclusions

Pancreatic body ,tail mass,biopsed

Recommendations

Endoscopist Dr Ahmed Ramadan

ENDOSCOPE
الاستقبال

التجمع الخامس المهندسين أكتوبر مصر الجديدة
info@endoscopeegypt.com
www.endoscopeegypt.com

endoscope

Endoscpoy

Date of Procedure 30/07/2026

Patient ID 32846

ENDOSCOPE

الدوسكوب لاجهزة الهضم والمناظير

Patient Images

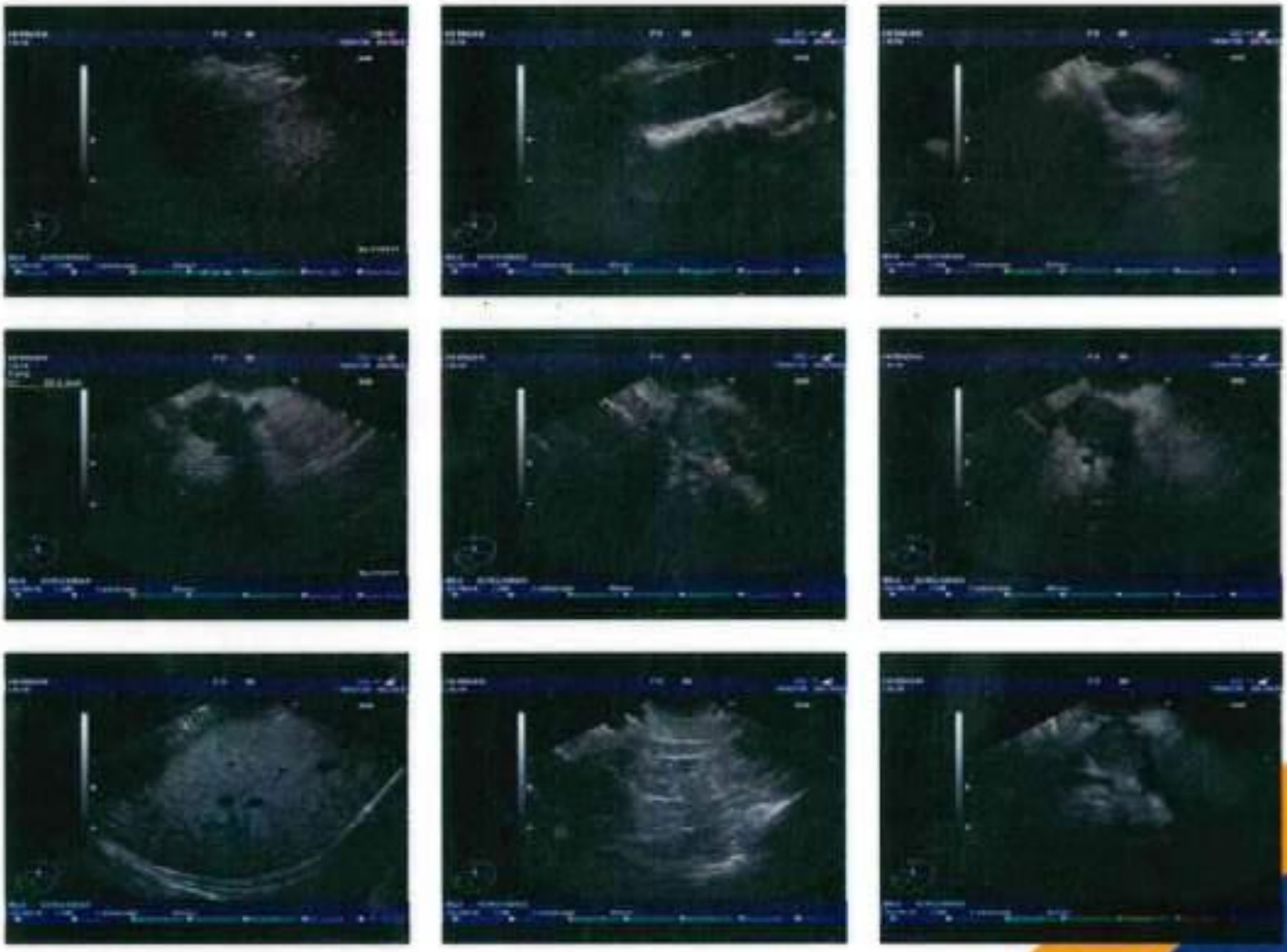
Patient Name حسام احمد خليل

Doctor Dr Ahmed Ramadan

Date of Birth 12-Dec-85

Referred By -

ENDOSCOPE الاستقبال



CT Scan

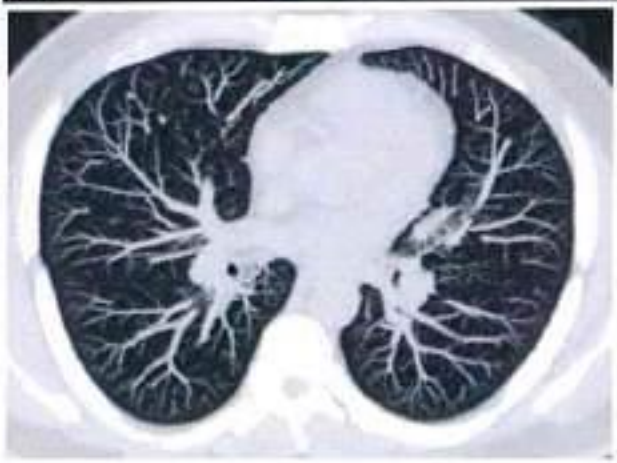
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SCAN

Name: Hossam Ahmed Khalil Khalil *ID: 2168429*
Referred by: Prof. Yasser Aldebeky *Date: 06/08/2026*

Multislice non contrast CT scan of the chest with multiplanar reformatting revealed:

- No pulmonary ground glass opacities, consolidation, cavitation.
- Few bilateral pulmonary nodules (more at the right); the largest seen at the middle lobe fissural based measuring 5 mm.
- Bilateral bronchial wall thickening.
- No pathologically enlarged hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Left thyroid lobe calcifications. for US assessment.
- Small sliding hiatus hernia.





OPINION:

- **Few bilateral pulmonary nodules. For follow-up.**
- **No current CT features of pneumonia.**

Dr. Mahmoud Mekhimar, MD

[[DOCTYPE:CT_SCAN]]
CT Scan

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LABS

Collection Date: 23/07/2026	**Patient Name:** الاستاذ/ حسام احمد خليل
Review Date: 23/07/2026	**Patient No.:** 3-43366
	Accession No.: 326077509
Referring Doctor: أ.د منجي المليجي	**Age:** 40Y

CHEMISTRY UNIT

Test Name	Result	Unit	Reference Interval
Creatinine, Serum	0.85	mg/dL	0.7 - 1.3

Reviewed By:
Dr.Sara Mohamed

Pt. Branch: المنيل - الروضه

Printed by: فرج4886بتر عيسى

A MEMBER OF ALFA MEDICAL GROUP
معامل الفا
16191

DR. AHMED YOUSSIF MONTASSER
Assoc. Professor of Pathology - TBRI
M.D. of Pathology, Cairo University
Fellow, Beaujon University Hospital, France
Member, European Society of Pathology
Member, USCAP
Member, ISUP

د. أحمد يوسف منتصر
أستاذ م الباثولوجي - معهد تيودور بلهارس
دكتوراة الباثولوجي - جامعة القاهرة
زميل مستشفى بوجون الجامعي - فرنسا
عضو الجمعية الأوروبية للباثولوجي
عضو الأكاديمية الأمريكية الكندية للباثولوجي

Name: حسام أحمد خليل | Age: 41 Y | Sex: Male

Referred by: **Prof. Dr. Ahmed Ramadan** | Signed out: 3-08-26 | Lab ID: 26AM2251

PATHOLOGY REPORT

Clinical Data: Pancreatic mass. **EUS:** Solid hypoechoic mass, 24x23mm at pancreatic body-tail junction.

Nature of Specimen: EUS-FNB.

Gross Appearance: Unstained films and hemorrhagic material entangling tan tissue fragments submitted for cellblock preparation.

Microscopic Features:

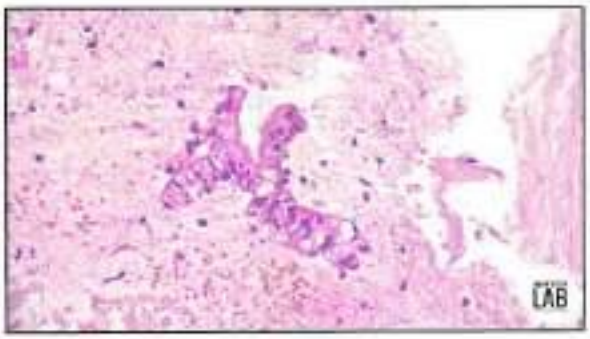
Cytologic examination of the films revealed hypocellular smears showing eosinophilic material entangling RBCs, inflammatory cells and rare groups of epithelial cells showing mildly atypical nuclei.

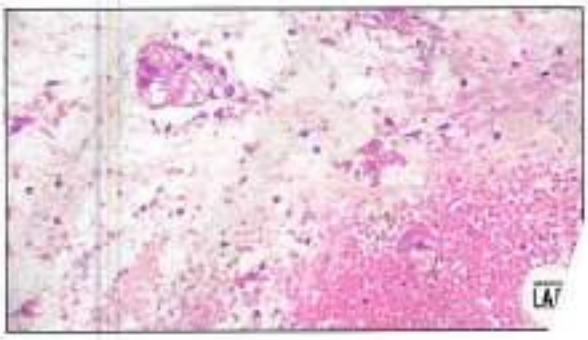
Sections examined from the cellblock prepared from the received aspirate revealed fibrinous material and blood clots entangling fragments of pancreatic tissue with marked fibrosing reaction, together with scattered groups and acini composed of mildly and moderately pleomorphic epithelial cells having large hyperchromatic nuclei, high nucleocytoplasmic ratio and vacuolated pale eosinophilic cytoplasm.

Opinion:

Pancreas, EUS-FNB,

Satisfactory Aspirate: Moderately Differentiated Adenocarcinoma. WHO Diagnostic Category: Malignant.





Signature

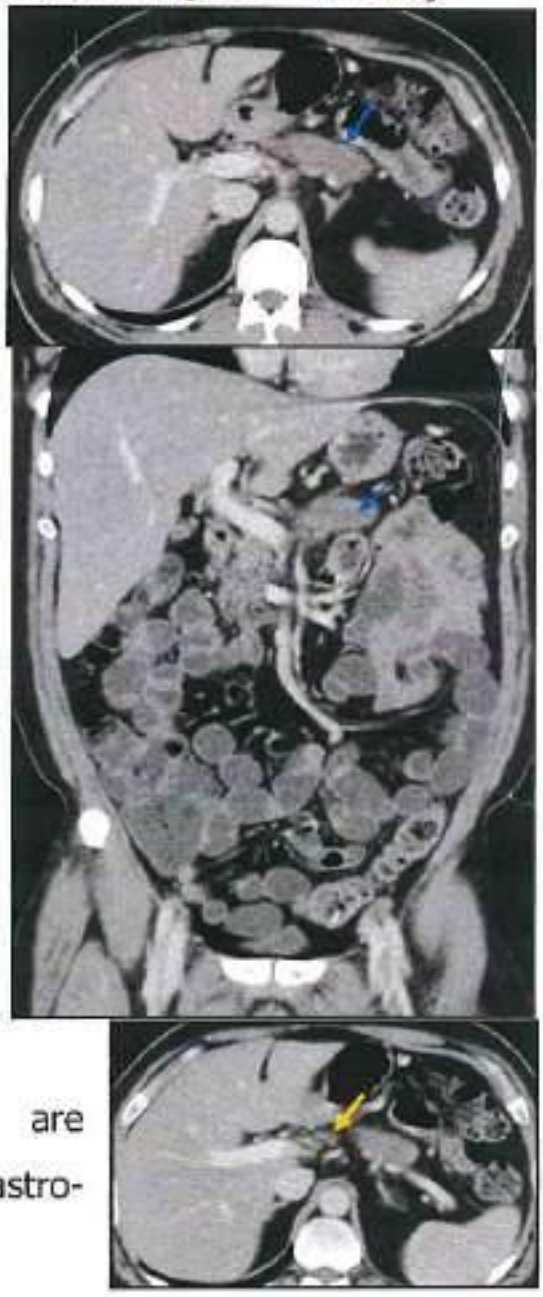
Ahmed Montasser, MD
ANATOMIC PATHOLOGIST

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Name: Hossam Ahmed Khalil Khalil *ID: 2168429*
Referred by: Prof. Mohamed Al Kassas *Date: 25/07/2026*

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:

- Swollen pancreatic body by the virtue of an ill-defined **(blue arrows)** hypodense area.
  - Associated smudging of the peri-pancreatic fat planes with pathologically enlarged peri-pancreatic lymph node **(yellow arrow)**, seen measuring about 0.9 cm in axial dimensions.
  - No gross pancreatic duct dilatation.
  - **Vascular relations show;**
    - Attenuation of the related proximal segment of the splenic artery.
    - Focal non-enhancing distal segment of the related aspect of the splenic vein, with patent rest of its segments. Mild stranding along the perivascular fat planes along celiac and splenic artery.
    - Dilated portosystemic collaterals are seen along the peri-gastric and gastro-splenic regions.



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CT Scan
{{DOCTYPE:CT_SCAN}}

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SCAN

- Mildly enlarged liver showing homogenous parenchymal attenuation. No sizable enhancing hepatic focal lesions or biliary radicles dilatation.
- Unremarkable CT appearance of the spleen, adrenals, both kidneys, aorta & IVC as well as pelvic organs.
- No sizable ascites.
- Scanned lung bases show right basal atelectatic bands.

OPINION:

- **Swollen pancreatic body by ill-defined hypodensity with related fat stranding, with focal splenic artery attenuation and distal splenic vein attenuation/non-enhancement as above described, for Doppler study correlation.**
  - **Diagnostic possibilities include inflammatory process (sequel of pancreatitis), however underlying neoplastic process cannot be excluded, for tumor markers correlation and further work-up.**
- **Mild hepatomegaly.**

Dr. Noha El Mansy, MD *Dr. Salma Hesham*

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CT Scan
{{DOCTYPE:CT_SCAN}}